28 October 2026 | Evaluation plan v5 | Discussion draft — no procurement approval
MEMORANDUM

Neonatal teleconsultation: five-option investment evaluation

Action required

Use the existing-evidence model to compare telephone, mobile teleconference, Panda Vision, ceiling/wall PTZ and a complete tablet/OBSBOT kit. Distinguish new investment from continuation or replacement. Review conditional results and feasibility before procurement or publication; no new trial or structured expert elicitation is required to conduct the model-based study.

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Presented by	CMO / neonatal, retrieval, virtual-care and finance partners

Executive summary

1. Decision and setting. Telephone-supported consultation and the RSQ/ANTS-NQ escalation pathway are retained in all arms. The pilot includes Atherton, Mareeba and Innisfail, with 10–15 consultations per site annually. The 4–5 Atherton episodes inform workflow, not efficacy or incidence. Use public historical births for the existing 33-site Queensland scenario, not as a current capability census. [ELC2026; USER2026; QH2022]

2. Complete configurations. ELC indicative installed costs are $45,000 for mobile teleconference, $10,000 for Panda Vision and $47,000 for fixed PTZ. The tablet/OBSBOT kit is a separate complete-system concept at an assumed $4,500, with $7,500 and wider alternatives tested. A $349 camera alone is not an equivalent service. Verify integration, mounting, cleaning, power, far-end control and total cost. No technical approval or invoice verification is implied. [ELC2026; M5]

3. Model and outcomes. Extend the existing severity-stratified cohort model, not the clinical pathways. Reference video effectiveness and delivery are equal by assumption, not demonstrated equivalence. Retain telephone fallback, critical transfers, rescue and local/receiving-hospital consequences. Report resource costs, family burden and actual budgets separately from NWAU × NEP and QWAU × QEP. Unverified QEP remains blank. Survival with major morbidity remains an exploratory extension, not an assumed primary benefit. [M2; M5]

4. Analyses now completed. Eighteen paired 20,000-draw PSA structures compare five options with shared systematic uncertainty. Twelve one-way dimensions and four two-way pairs cover pilot/state decisions. Exact 3,125-point five-way and 2,401-point equipment grids, independent 12-dimensional designs and Sobol variance analyses address interactions. Existing v4 scenarios and survival analyses are retained. Deterministic design frequencies are not probabilities. [M5]

5. What the results imply. Under equal-video/new-purchase assumptions, the proposed kit has 53.9% probability of being cost-optimal; Panda has 46.0% probability of saving resources versus telephone but is never the cheapest video choice. Retaining installed Panda instead makes Panda cost-optimal in 61.4% of draws. With uncertain prototype cost/delivery, pilot probabilities become telephone 51.0%, Panda 27.2% and kit 21.8%; the state expected-value choice switches to Panda. These are conditional model results, not clinical rankings. [M5]

6. Interpretation and publication. Input-only clustering was weak and unstable; do not infer patient groups. A decision-focused classifier achieved 95.0% holdout agreement, but its worst annual pilot regret was $8,970; exact model evaluations remain authoritative. First journal target: Journal of Telemedicine and Telecare. Complete critical full-text source verification, independent model review, a clean reproducible release and governance/authorship/data-rights statements before submission. [M5; JTT]

Engagement and consultation	
Consumers	Report family costs and separation; no consumer-derived utility or disability-based exclusion is imposed.
Clinicians and staff	Use existing clinical and operational knowledge. Formal elicitation and new prospective recruitment are not prerequisites.
External stakeholders	No Queensland Health, RSQ, neonatal-network or vendor endorsement of this comparative model is claimed.

Implications	
Budget	Separate full procurement cash, annualised resource cost, sunk capital and cross-service attribution. An allocation fraction does not reduce the purchase bill. Prototype prices are unquoted.
FTE	Readiness, training, specialist support and network capacity remain model assumptions. No new positions or funding allocation are approved.
Risk	Sparse device-specific evidence; assumed common video effects; historical denominators; uncertain marginal retrieval cost and prototype reliability. Null/harm and eligibility-restricted comparisons are explicit.
Legal	Existing aggregate-evidence model. Confirm governance, coauthor agreement and rights before any linked records, public code/data release or submission. No exemption or approval is invented.
ICT	Panda Vision and OBSBOT are not interchangeable labels: the latter arm is a complete tablet-based alternative. Establish technical eligibility before interpreting a cost-optimal choice as procurable.
Impact on First People's Health	Preserve rural access and family-displacement consequences. Cost saving alone is not the decision criterion; extra survival may appropriately increase future care expenditure.

Strategic Plan alignment
Our care: appropriate specialist support. Our people: usable, reliable systems. First Peoples health: equitable access. Our sustainability: transparent procurement, resource costs and uncertainty.

Attachment/s
1. Comparative supplement and publication strategy v5. 2. Results workbook, full CSV tables and 15 PNG/SVG figures. 3. CHHHS presentation v5. 4. Repository, Git history, Conductor tracks and actual assurance receipts. M2/M5 denote model outputs, not external observations; source registers identify all citations. Full connected-toolchain and hosted-CI acceptance remain incomplete.